

Prescription Commerce Commercialization Strategy

A pharmacy-owned digital prescription intake, quotation, payment, pickup, and delivery operating layer.

EXECUTIVE DECISION

Build the category around the pharmacy, not a single hospital

The hospital-gate opportunity is a strong pilot wedge. The product itself should serve prescription demand from any approved source.

Recommendation**PURSUE**

Proceed now with a bounded design-partner pilot, while keeping production launch conditional on operational, privacy, and regulatory readiness.

Commercial thesis**B2B2C**

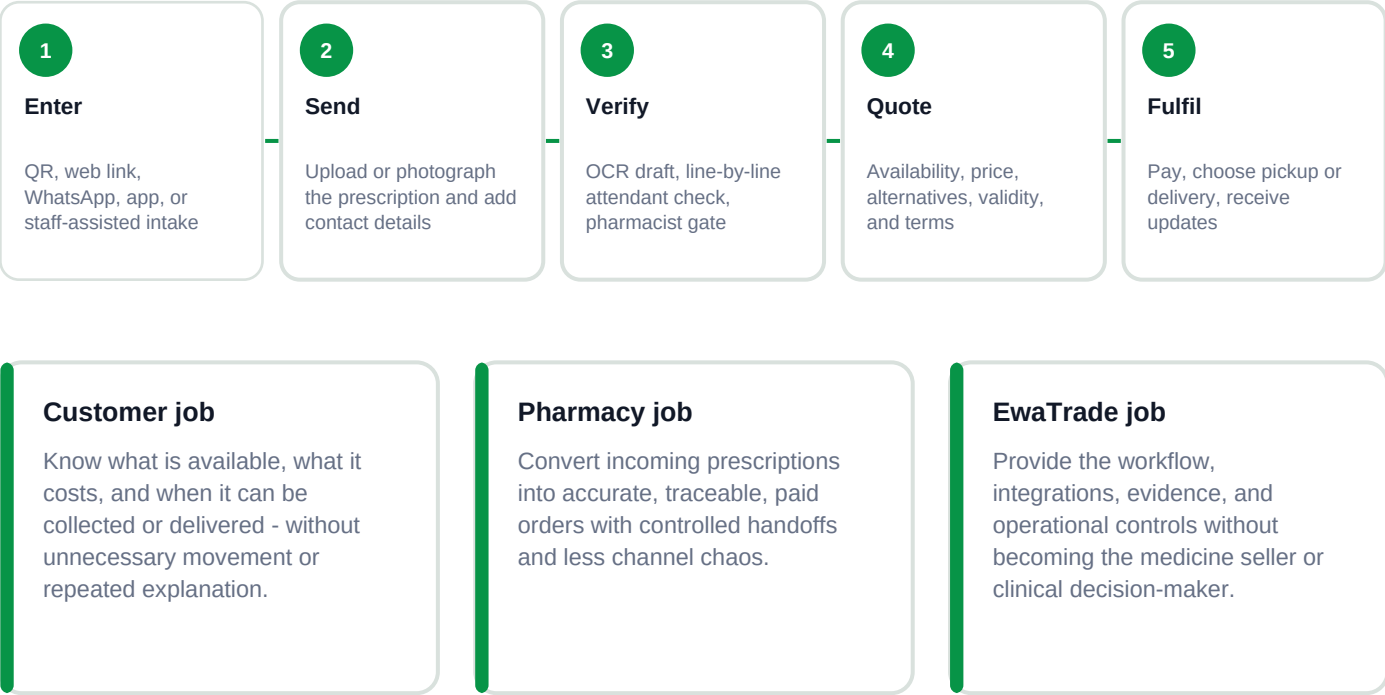
Charge the pharmacy for workflow software and measurable outcomes. Keep medicine sales, clinical judgment, and dispensing with the licensed pharmacy.

- Do not build a standalone consumer marketplace first. Start with QR, web link, WhatsApp, and staff-assisted intake.
- Use OCR only to create an editable transcription. An attendant checks every line; a licensed pharmacist controls clarification, substitution, approval, and release.
- Monetize through implementation, recurring branch subscription, completed-order fees, and optional delivery coordination margin.
- Replace every price, conversion, cost, and target hypothesis with baseline and pilot evidence before commercial rollout.

PRODUCT THESIS

One managed route from prescription to fulfilment

EwaTrade turns fragmented calls, chats, walk-ins, and paper notes into a visible pharmacy workflow.



MARKET ENTRY

Demand can originate anywhere the pharmacy earns trust

The product is channel-agnostic; the pharmacy decides which sources and delivery lanes it supports.

Hospital and clinic

QR posters, discharge desks, outpatient units, referral cards

Doctors and diagnostics

Secure link shared after consultation or test results

Pharmacy channels

Website, social pages, Google profile, call centre, storefront QR

Repeat and chronic care

Refill request, caregiver ordering, saved contact and preferred fulfilment

Distant demand

Approved service zones, manual delivery quote, restricted-item exclusions

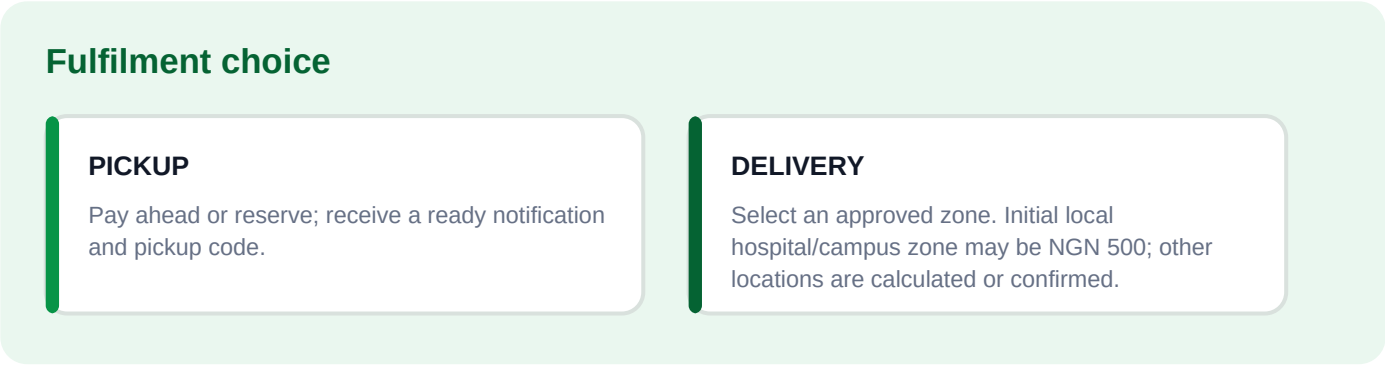
Walk-in recovery

Staff capture queues, out-of-stock follow-up, quote now and collect later

CUSTOMER EXPERIENCE

The customer flow stays simple even when operations are complex

QR scan opens a choice: continue online or continue in the pharmacy's WhatsApp chat.



The NGN 500 hospital/campus fee is an initial pilot configuration, not a universal product promise.

Customer, transcription, and pharmacist controls

Prescription commerce is a controlled workflow, not just an upload form.

Customer intake

- QR, shareable link, responsive web
- Continue online or WhatsApp
- Image/PDF upload and capture guidance
- Contact, source, consent, and fulfilment intent
- Status page and notifications

OCR and transcription

- Image-quality check and clearer-image request
- Line extraction with confidence indicators
- Mandatory line-by-line attendant verification
- Edit history, timestamps, and reviewer identity
- Original image retained under approved policy

Pharmacist control

- Clinical clarification queue
- Prescription validity and restricted-item gate
- Substitution/alternative approval
- Final quote and release authority
- Exception and rejection reasons

FEATURE SYSTEM II

Quote, payment, fulfilment, and management

Each request has a visible owner, status, promise, and audit trail.

Quote and order

- Full, partial, unavailable, clarification states
- Item, quantity, price, total, validity
- Versioned quote and acceptance
- Payment link, receipt, reconciliation
- Cancellation and refund controls

Pickup and delivery

- Packing queue and ready status
- Pickup code and authorised collector
- Configurable zone fees and eligibility
- Manual or calculated distant-location fee
- Courier assignment, proof, failure handling

Operations and analytics

- Role-based staff inbox and SLA timers
- POS/inventory/accounting integration path
- Templates, customer updates, escalations
- Branch/channel/source performance
- Conversion, stock, turnaround, reliability

AI BOUNDARY

AI transcribes; people remain accountable

The design must reduce typing without automating clinical judgment.

1. Capture

Customer submits image or PDF. The system checks legibility and asks for a clearer image when needed.

2. Extract

OCR produces a draft list of lines and highlights low-confidence text. It does not create a clinical order.

3. Verify

The attendant compares every generated line to the original and confirms or corrects it before proceeding.

4. Approve

A licensed pharmacist resolves clinical ambiguity, restricted items, and substitution, then releases the quote.

Non-negotiable release rule

No quote proceeds from raw OCR output. Verification is line by line, recorded, and followed by the pharmacy's licensed review and dispensing controls.

COMMERCIAL ARCHITECTURE

A hybrid model aligns recurring software value with completed orders

Commercial terms below are hypotheses for customer discovery and pilot negotiation.

Implementation**NGN 250k-750k**

Configuration, workflow mapping, training, launch support, and initial integrations.

Branch subscription**NGN 100k-300k / month**

Workflow workspace, roles, dashboards, support, and standard channel operations.

Completed order**NGN 100-300**

Fixed fee only when a prescription request becomes a paid medicine order.

Delivery coordination**10%-20% margin**

Optional margin on the customer delivery fee where EwaTrade coordinates dispatch.

Premium services**Quoted separately**

Additional integrations, SLA, analytics, storage, messaging, and multi-branch controls.

Customer charge**Pharmacy-controlled**

Medicine price and delivery fee remain transparent and approved by the pharmacy.

ILLUSTRATIVE ECONOMICS

A decision model, not a revenue claim

Base-case figures below demonstrate the mechanics for one branch and must be replaced with pharmacy and pilot data.

Requests / month**1,500**

Illustrative base scenario

Paid orders**675**

Illustrative base scenario

Monthly revenue**NGN 333,573**

Illustrative base scenario

Direct cost**NGN 132,500**

Illustrative base scenario

Contribution**NGN 201,073**

Illustrative base scenario

Contribution margin**60.3%**

Illustrative base scenario

- The model includes a monthly subscription, completed-order fee, delivery coordination margin, and setup fee amortised over 12 months.
- Direct cost includes illustrative OCR, messaging, storage/security, and branch support allowances.
- The workbook contains Conservative, Base, and Growth scenarios; a 24-month branch ramp; formula checks; and pilot data capture.
- Do not approve pricing from this page. Validate willingness to pay, basket value, pharmacy margin, channel volume, support workload, and delivery economics first.

GO-TO-MARKET

Win one design partner, prove the workflow, then sell a repeatable operating system

The first pharmacy is a learning partnership and reference case, not the definition of the market.

1

Design partner

Executive sponsor, branch owner, pharmacist lead, customer service, finance/POS, privacy/legal, and dispatch owner.

2

Active test

Baseline observation, prototype walkthrough, shadow-mode transcription, controlled live orders, daily issue review.

3

Proof package

Measured conversion, turnaround, readiness, staff workload, customer trust, safety, and privacy evidence.

4

Repeatable sale

Segment playbook for hospital-adjacent, urban delivery, multi-branch, and chronic-care pharmacies.

PILOT SCORECARD

Commercial conversion is primary; speed and reliability must support it

Targets are provisional gates. Guardrails override growth metrics.

PRIMARY	Quote-to-paid conversion	>= 50% hypothesis
PRIMARY	Median quotation turnaround	<= 10 min hypothesis
PRIMARY	Ready-on-promise rate	>= 95% hypothesis
DRIVER	Image clarification	Measure friction
DRIVER	OCR line correction	Measure workload
DRIVER	Full availability	Measure stock coverage
DRIVER	Delivery selection	Size dispatch demand
GUARDRAIL	Safety/privacy/pharmacy errors	Zero tolerance

EVIDENCE PLAN

Collect the facts that determine product, pricing, and authority

No patient names, prescription images, phone numbers, diagnoses, or medicine details belong in the commercial pilot workbook.

Demand

Requests by day/hour and channel; average lines; source mix; abandonment reasons.

Commercial

Average basket, pharmacy gross margin, lost-sale value, willingness to pay, approval path.

Operations

Current quote time, staff minutes, availability outcomes, packing and pickup wait.

Delivery

Zone demand, fees, courier payout, distance, failure rate, eligibility and cold-chain rules.

Customer

Web vs WhatsApp preference, trust, payment preference, delivery willingness.

Technical

POS/inventory/accounting systems, WhatsApp provider/number, devices, connectivity, access.

Compliance

PCN role/licence pathway, privacy roles, retention, processor terms, delivery SOP.

Risk

Misread text, impersonation, leakage, incorrect fulfilment, controlled items, downtime.

GOVERNANCE

The pharmacy remains the regulated clinical and commercial operator

EwaTrade provides technology and workflow controls. Final structure must be confirmed with pharmacy leadership, PCN guidance, and Nigerian privacy counsel.

Pharmacy owns

Medicine catalogue and price; professional review; substitution; dispensing; sale; controlled-item policy; packing; customer promise; refund authority.

EwaTrade provides

Intake, transcription assistance, role-based workflow, quote/order/payment orchestration, messages, fulfilment coordination, reporting, and audit evidence.

- Confirm the applicable electronic-pharmacy licence and role arrangement before production launch.
- Document controller/processor responsibilities, lawful basis, consent where required, retention, access, deletion, incident response, and subprocessor terms.
- Use managed private object storage, time-limited access, encryption, least privilege, audit logs, backups, and tested recovery for prescription media.
- Define explicit delivery exclusions and escalation for cold-chain, restricted, controlled, or clinically unsuitable items.
- Treat legal interpretation and implementation guidance as launch dependencies, not assumptions hidden inside the product.

ROADMAP

Advance through evidence gates, not feature volume

The workforce commitment enables speed; governance and operating truth determine readiness.

0-2 weeks**Discovery and authority**

Sponsor alignment, baseline, workflow map, data map, licence/privacy questions, commercial hypotheses.

3-6 weeks**Concierge MVP**

QR/web/WhatsApp intake, OCR draft, verification, pharmacist quote, pickup, fixed local zone, manual other-zone quote.

7-10 weeks**Controlled live pilot**

Payments, notifications, operations dashboard, service levels, de-identified metrics, daily exception review.

11-14 weeks**Decision and hardening**

Pricing decision, ROI case, security controls, SOPs, integrations, delivery policy, support model.

After gate**Repeatable rollout**

Multi-branch configuration, partner onboarding, channel kits, analytics, approved service lanes.

DECISION GATES

What must be true before expanding beyond the pilot

A failed guardrail stops rollout even when demand is strong.

DESIRABILITY

Customers understand and trust remote quotation; a meaningful share of approved quotes becomes paid orders.

VIABILITY

The pharmacy sees measurable incremental or retained value; pricing clears support and variable costs.

FEASIBILITY

Staff can verify, quote, pack, and hand off within target workload and service levels.

SAFETY

Pharmacist controls, restricted-item rules, and release accountability operate without bypass.

PRIVACY

Data roles, access, storage, retention, deletion, processor terms, and incident response are approved.

REPEATABILITY

Configuration and onboarding can be repeated without bespoke rebuilding for each pharmacy.

SOURCES AND CAVEATS

Evidence used - and what it does not prove

Official sources frame the regulatory questions. Category examples validate customer behaviour only; they do not prove EwaTrade demand or performance.

Pharmacy Council of Nigeria - Electronic Pharmacy Regulations 2026

<https://pcn.gov.ng/wp-content/uploads/2026/04/Electronic-Pharmacy-Regulation-2026-B81-108.pdf>

Pharmacy Council of Nigeria - Publications

<https://pcn.gov.ng/about-pharmacy-council-nigeria/publications/>

Nigeria Data Protection Act 2023

https://ndpc.gov.ng/wp-content/uploads/2024/03/Nigeria_Data_Protection_Act_2023.pdf

Drugstore.ng - category workflow example

<https://drugstore.ng/>

Sanlive Pharmacy - prescription upload example

<https://sanlivepharmacy.com/upload/prescription>

Amkamed - WhatsApp prescription workflow example

<https://www.amkamed.com/our-services>

Pricing, volumes, costs, conversion targets, timelines, and pharmacy ROI are hypotheses. They are included to make validation concrete, not to represent committed commercial terms or forecast performance.